CV Protocol

Setup: 0530

- Heart pack has all anesthesia contents you need for setup
- Heart box has all heart drugs.
- Make blood set with NS, hotline tubing, extension set and heplock. Go ahead and connect into fluid warmer.
- Make NS infusion with multi- port and micro drip tubing (will connect to white port of swan).
- Set up pressure bag and triple transducer. Place in transducer holder at head of bed (from left to right: arterial, PA, CVP) and flush all lines.
- Connect to unsterile end of swan, flush all ports, and place under head of bed.
- Connect CO/CI and Sv02 cables. Connect art, PA, CVP cables and zero all lines.
- Mix and setup Norepi into the “B” channel. Setup Cleviprex into the “C” channel (These will be on the same 4-pump IV pole to go with patient to ICU).
- Have Epi drip ready for patients with EF <40% or IABP/Impella
- Amicar 10g in 50ml bag setup into “B” channel of second IV pump (Mike does 5gm in 50ml).
- Right side 1000cc LR to be connected to Cordis (Kevin and Lisa)
- Prepare syringes: Propofol, Zemuron, Anectine (if appropriate), Versed, Sufenta, Heparin
- Have Levophed and Cleviprex pushes ready
- Setup Casmed Foresight sensors
- Enter patient info, height, weight, and H&H into CO/CI box. Complete in-vitro calibration
- Unhook art line tubing from triple transducer to take to preop.
- Have CASS ETT ready
- Connect foam electrodes to 5-lead

PRE CPB:

-start arterial line in holding before 7am

-antibiotics: Vancomycin for valve cases/Ancef otherwise with the exception of allergies

-RN in OR places additional peripheral line in OR (blood line) post induction; fluid warmer on this line

-TEE (MDA or CRNA)

-placed cordis and swan with SG (sterile gown/gloves); perfusionist draws blood from line (initial ACT)

-ABG

-Amicar 10GM infusion/or push bolus

-Begin Amicar 1GM/HR infusion to run throughout case entirety

-Calculate Heparin dose (300 U/kg)

-Connect and flush pressure line off CVP port, passed off from scrub tech

-Heparin dose as requested per surgeon

-ACT draw 3 minutes post Heparin

-maintain SBP 80-90 mmHg for aortic cannulation

-Use CVP (not PA) for retrograde line

-record PRE CPB UOP; inform perfusionist

-after initiation CPB, stop all gases and IVF's except Amicar and background NS infusion

CPB:

-prepare re-dose antibiotic

-prepare Protamine infusion (10mg for every 100u of Heparin given)

-infuse meds as needed per request of perfusionist

-record total UOP

POST CPB:

-restart swan- CO/CI

-expand lungs manually as requested per surgeon

-restart gases and IVF's

-radio MDA to notify off CPB and TEE

-(Lee MD only): EPI 0.02 mcg/kg/min to come off CPB

-Systolic below 100 for aortic decannulation

-Protamine infusion start: ONLY AFTER REQUESTED PER SURGEON tell Perfusion when half finished

-draw ACT and ABG: Quicknote

-redose antibiotic

-infuse cell saver blood and platelet poor plasma

-record UOP

-RN will call report to ICU report; transport pt with lines to side that pump will be in OR -return from ICU with IV pump x4 modules, SvO2 cable, pressure line manifold and pacemaker if you give the SICU one.